

1. Administrative & Study Identification

Official Study Title: A Randomized, 2-Arm, Phase 3 Study of Elranatamab (PF-06863135) Versus Lenalidomide in Patients With Newly Diagnosed Multiple Myeloma After Undergoing Autologous Stem-Cell Transplantation **Brief Title:** Study With Elranatamab Versus Lenalidomide in Patients With Newly Diagnosed Multiple Myeloma After Transplant (MagnetisMM-7) **Protocol Number:** C1071007 **NCT Number:** NCT05317416 **Sponsor/Company Name:** Pfizer **Investigational Product:** Elranatamab (PF-06863135) and Lenalidomide (Revlimid, ATC Code: L04AX04) **Phase of Development:** PHASE3 **Trial Status:** RECRUITING **Disease Area:** Multiple Myeloma **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate whether elranatamab monotherapy can provide clinical benefit compared to lenalidomide monotherapy (control) in participants with newly diagnosed multiple myeloma (NDMM) after undergoing autologous stem cell transplant (ASCT).

2.2 Background & Rationale Elranatamab is a bispecific antibody: binding of elranatamab to CD3-expressing T-cells and BCMA-expressing multiple myeloma cells causes targeted T-cell-mediated cytotoxicity. While lenalidomide is the standard maintenance therapy post-ASCT, many patients eventually relapse. This study addresses the medical necessity of optimizing post-ASCT maintenance to eliminate residual disease and improve survival outcomes.

2.3 Summary of Study Structure In Part 1 and Part 2 of the study, participants will either receive elranatamab (Arm A and Arm C) as an injection under the skin at the study clinic or lenalidomide (Arm B) orally once daily at home. Participation in the study will be approximately five years.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Lenalidomide) **Blinding:** Open-label **Randomization:** 2-Arm Design (1:1 Allocation)

3.2 Planned Enrollment & Duration Targeted Enrollment (\$N\$): 760 **Estimated Study Start:** 25-Mar-2022 **Estimated Primary Completion:** 04-Aug-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Diagnosis of Multiple Myeloma as defined according to International Myeloma Working Group (IMWG) criteria with measurable disease at diagnosis. 2. Part 1 patients must be MRD positive; Part 2 patients can be MRD negative or MRD positive. 3. History of induction therapy for newly diagnosed MM, followed by high dose therapy and autologous stem cell transplant (ASCT). Randomization must occur within 120 days from the stem cell transplant. For participants who receive consolidation therapy after ASCT, randomization must occur within 60 days of consolidation and within 7 months from ASCT. 4. Partial Response or better according to IMWG criteria at the time of randomization. 5. Must have an archival bone marrow aspirate sample(s) to identify the dominant malignant (index) clone by central laboratory NGS test (ClonoSEQ assay) that is used to track MRD status. This sample should preferably be collected before induction treatment (eg, at diagnosis) or before transplant. 6. ECOG performance status ≤ 1 . 7. Resolved acute effects of any prior therapy to baseline severity or CTCAE Grade ≤ 1 . 8. Not pregnant and willing to use contraception.

4.2 Exclusion Criteria 1. Plasma cell leukemia, Amyloidosis, Waldenström's macroglobulinemia, or POEMS syndrome. 2. Known active CNS involvement or clinical signs of myelomatous meningeal involvement. 3. Previous MM maintenance treatment or prior treatment with BCMA targeted therapy. 4. Any other active malignancy within 3 years prior to enrollment, except for adequately treated basal cell or squamous cell skin cancer, or carcinoma in situ. 5. Active, uncontrolled bacterial, fungal, or viral infection, including (but not limited to) HBV, HCV, and known HIV or AIDS-related illness. 6. Previous administration with an investigational drug or vaccine within 30 days (or as determined by the local requirement) or 5 half-lives preceding the first dose of study intervention used in this study (whichever is longer).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * *Experimental Arms (Arm A and C):* Elranatamab administered via subcutaneous (SC) injection under the skin at the study clinic. * *Active Comparator Arm (Arm B):* Lenalidomide taken orally once daily at home.
Duration of Treatment: Participation in the study is approximately 5 years, or until disease progression/unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications for symptom and adverse event management. **Prohibited:** Previous administration of an investigational drug or vaccine ≤ 30 days (or 5 half-lives).

Other MM maintenance treatments or concurrent BCMA-targeted therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Pre-Randomization	Informed Consent, IMWG response assessment, MRD status check via bone marrow aspirate (ClonoSEQ NGS), ECOG evaluation
Baseline	Day 0	Randomization, First Dose
Interim	Weekly/Bi-weekly	Clinic visits for SC injections (Elranatamab arms), Efficacy/Response assessment, Safety Labs
End of Study	~Year 5	Final survival/PFS follow-up, Exit Interview, IP Accountability

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes

- Minimal Residual Disease negativity rate:** Minimal Residual Disease negativity rate per IMWG criteria as assessed via Next Generation Sequencing.
- Progression Free Survival:** Progression Free Survival assessed by Blinded Independent Central review per IMWG response criteria.

7.2 Secondary Outcomes

- Overall Survival**
- Sustained MRD negative rate**
- Progression Free Survival**

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via patient clinic visits, laboratory evaluations, and physical examinations. Acute effects of prior therapies must have resolved to baseline or CTCAE Grade ≤ 1 prior to enrollment.

Serious Adverse Events (SAEs): Required to be reported to the sponsor within 24 hours of site awareness.

Data Monitoring Committee (DMC): Independent Data Monitoring Committee (IDMC) to review ongoing safety and efficacy data for the Phase 3 open-label trial.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy analyses; Safety Set for all safety and tolerability endpoints.

Sample Size Justification: Target $N=760$ appropriately powered to evaluate clinical benefit for elranatamab versus lenalidomide.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and centralized remote monitoring. **Audit Trail:** Data tracking via centralized eCRF systems, specifically managing central laboratory NGS testing (ClonoSEQ assay) integrations.

11. Study Identifiers & Governance

Primary ID: C1071007 **Registry Number:** NCT05317416 **Sponsor/Lead Organization:** Pfizer **Official Regulatory Title:** A Randomized, 2-Arm, Phase 3 Study of Elranatamab (PF-06863135) Versus Lenalidomide in Patients With Newly Diagnosed Multiple Myeloma After Undergoing Autologous Stem-Cell Transplantation

12. Clinical Framework (Multiple Myeloma Specific)

Primary Condition: Newly Diagnosed Multiple Myeloma (NDMM) post-ASCT **Medical Methodology:** Bispecific Antibody (CD3/BCMA) vs. Standard Immunomodulator **Optimization Target:** Eradication of residual disease (MRD negativity rate) and prolongation of PFS

13. Study Procedures & Methodology

Management Pathway: Post-ASCT maintenance therapeutic pathway **Education Protocol (HCP):** Subcutaneous injection protocol training and step-up dosing schedule management **Education Protocol (Patient):** Oral lenalidomide daily administration adherence at home vs. frequent clinic visits for SC injections **Quality Audit Mechanism:** Centralized laboratory NGS testing (ClonoSEQ assay) for unified MRD tracking

14. Observation & Follow-Up Schedule

Total Duration: Approximately 5 years **Remote Monitoring Frequency:** Periodic continuous tracking based on local clinic guidelines for oral lenalidomide arm at home **Checklist Review Interval:** Routine at every dosing/assessment cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				